

Buprenorphine MAT Care Guide Full Checklist – Primary Care

1. Patient Identification & Initial Assessment

- ☐ Confirm DSM-5 diagnosis of Opioid Use Disorder (OUD)
- ☐ Assess severity (mild/moderate/severe)
- ☐ Review medical history
 - Hepatic disease, respiratory disease, pregnancy
 - Chronic pain conditions
- ☐ Review psychiatric history
 - Depression, anxiety, bipolar disorder
 - PTSD, suicidality
- ☐ Assess social stability
 - Housing
 - Transportation
 - Employment/legal issues
- ☐ Evaluate readiness for treatment and patient goals

2. Informed Consent & Treatment Agreement

- ☐ Review risks, benefits, and alternatives to buprenorphine
- ☐ Explain expectations for treatment structure and monitoring
- ☐ Discuss: Risk of diversion

Overdose risks with alcohol/benzodiazepines

Importance of adherence

- ☐ Obtain written informed consent
- ☐ Review and sign Substance Use Disorder

Treatment Contract

Includes:

Single prescriber/pharmacy
Urine drug screening expectations
Pill counts
PDMP monitoring
Safe storage requirements
Grounds for treatment modification or escalation

- ☐ Document consent and contract in EHR

3. Baseline Testing & Documentation

- ☐ Urine Drug Screen (UDS) (baseline)
 - Opioids
 - Buprenorphine (if already taking)
 - Benzodiazepines
 - Fentanyl (if available)
 - Other substances per clinic protocol
- ☐ Pregnancy test if applicable
- ☐ Baseline labs (as clinically indicated):

- CMP/LFTs
- Hepatitis C screening
- HIV screening
- ☐ Baseline PDMP review
- Document findings

4. Induction & Dosing Plan

- ☐ Confirm patient is in mild–moderate opioid withdrawal before induction
- ☐ Induction plan documented (office-based or home induction)
- ☐ Initial dose prescribed per protocol
- ☐ Clear written and verbal induction instructions provided
- ☐ Naloxone prescribed and education provided to patient and family

5. Ongoing Visit Monitoring (EVERY VISIT)

- ☐ Urine Drug Screen – Standing Protocol
 - Performed at every visit
 - Document results and clinical interpretation
 - Address unexpected results in a non-punitive manner
- ☐ PDMP Review
 - Review at every visit
- Document:
 - Other opioid prescriptions
 - Benzodiazepines
 - Early refills or multiple prescribers

- ☐ Medication Adherence & Safety
- ☐ Review dose and adherence
- ☐ Assess cravings and withdrawal symptoms
- ☐ Assess side effects
- ☐ Confirm safe storage
- ☐ Pill/Film Counts
- ☐ Conduct routine and/or random pill counts
- ☐ Document discrepancies and response plan if concerns arise
- ☐ Clinical Assessment
- ☐ Ongoing substance use assessment
- ☐ Mental health symptom check
- ☐ Functional improvement (work, relationships, stability)
- ☐ Overdose risk assessment

6. Prescription Management

- ☐ Prescribe appropriate quantity based on stability
 - Weekly/biweekly → monthly as appropriate
- ☐ No early refills without documented clinical rationale
- ☐ Single pharmacy verified
- ☐ Adjust dose only after clinical evaluation

7. Behavioral Health & Psychosocial Support

- ☐ Encourage community recovery support
 - Narcotics Anonymous (NA)
 - SMART Recovery (if patient prefers non-12-step)

- ☐ Provide local meeting lists or online options
- ☐ Refer to:
 - Individual or group therapy
 - Substance use counseling
- ☐ Psychiatry referral for:
 - Co-occurring mental health disorders
 - Diagnostic uncertainty
 - Complex polypharmacy
- ☐ Document patient engagement or barriers to engagement

8. Care Coordination & Follow-Up

- ☐ Establish follow-up interval based on stability
- ☐ Coordinate with:
 - Behavioral health
 - Psychiatry
 - Case management/social work
- ☐ Address social determinants (transportation, housing, insurance)
- ☐ Update treatment plan at regular intervals

9. Managing Concerning Behaviors (Non-Punitive Approach)

- ☐ Unexpected UDS results addressed with patient
- ☐ Missed appointments reviewed
- ☐ Diversion concerns assessed clinically
- ☐ Treatment intensification considered before discontinuation:
 - More frequent visits

- Increased monitoring
- Behavioral health escalation
- ☐ Documentation clearly reflects clinical decision-making

10. Maintenance, Tapering, or Transition Planning

- ☐ Regularly review long-term treatment goals
- ☐ Tapering only patient-driven and clinically appropriate
- ☐ If discontinuing:
 - Gradual taper plan
 - Alternative supports arranged
 - Overdose risk counseling provided
- ☐ Warm handoff if transferring care

Optional Add-Ons for EHR Integration

Standing UDS order
 PDMP auto-documentation template
 MAT smart phrases
 Pill count documentation tool
 NA/therapy referral order set